Tunzuri User Manual

Hospital management system — a guide for facility staff and administrators

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Contents

About this manual

Getting started

- Signing in
- Finding your way around
- Your profile and password
- Who can do what

A patient's journey

Reception

- Registering a patient
- Finding a patient
- Opening a visit
- Appointments

Queues

Triage

Consultation

- Notes and diagnosis
- Ordering lab tests
- Ordering imaging
- Prescribing
- Admitting a patient
- Completing the visit

Laboratory

Radiology

Pharmacy

- Dispensing a prescription
- Watching the pharmacy shelf
- Restocking the pharmacy

Cashier

- Shifts
- Taking a payment
- Waivers

Insurance and corporate accounts

- Schemes
- Preauthorisations
- Claims

Wards

- The ward board
- Caring for an inpatient
- Discharge

Maternity

Theatre

Inventory and stock

- How stock moves
- Setting up your items
- Adding stock without a purchase order
- Buying from suppliers
- Requisitions: moving stock to a department
- Returning stock
- Adjustments: expiry, damage, loss and counts
- Stock levels
- The stock card: an item's full history
- Stock usage: what each department and employee used

Reports

Administration

- Users
- Departments
- Wards and beds
- Price list
- Audit trail

Setting up a new facility

Platform administrator

Troubleshooting and common questions

Glossary

About this manual

Tunzuri runs a facility's whole patient journey in one place: registration, queues, triage, consultation, laboratory, imaging, pharmacy, wards, maternity, theatre, cashier, insurance, stock and Ministry of Health reports.

This manual is for the people who use Tunzuri every day: receptionists, nurses, clinicians, laboratory and imaging staff, pharmacists, storekeepers, cashiers and the facility administrator. Each chapter covers one area of work. Read the chapters for the screens you use, plus **Getting started**.

Conventions used in this manual

- **Bold text** is a button, field or menu item as it appears on screen.
- Amounts are in Uganda shillings (UGX).
- "Your administrator" means the person who runs Tunzuri for your facility.

Getting started

Signing in

1. Open your facility's Tunzuri address in a web browser (Chrome, Edge, Firefox or Safari).
2. Enter your email address and password, then select **Log in**.
3. If you were given a temporary password, change it straight away (see *Your profile and password* below).

If you forget your password, select **Forgot your password?** on the sign-in page and follow the link sent to your email.

For safety, Tunzuri signs you out automatically after 15 minutes without activity. Anything you have not saved is lost, so save forms before stepping away.

Finding your way around

The menu on the left shows only the areas you have access to. They are grouped as:

Group	Screens
Main	Dashboard, Patients, Appointments
Clinical	Queues, Laboratory, Radiology, Wards, Maternity, Theatre
Operations	Pharmacy, Cashier, Insurance, Inventory, Stock levels, Stock usage, Reports
Administration	Users, Departments, Wards & beds, Price list, Audit trail

If you need a screen you cannot see, ask your administrator to give you that area.

Searchable lists. Long lists, such as items, medicines, suppliers, employees, beds and visits, are search boxes rather than plain dropdowns. Click the box and type part of the name. Matches appear as you type. Use the arrow keys and **Enter**, or click a match, to choose it. The list shows extra detail (unit, price, ward) on the right of each match.

Your profile and password

Select your name (at the top right, or at the bottom of the menu) to open your profile. Here you can update your name and email, and change your password. Use a password of at least eight characters that you do not use anywhere else.

Who can do what

Your administrator gives each person one or more *areas*. Each area opens a set of screens.

Area	What it covers
Reception	Patients, appointments, queues and the cashier

Area	What it covers
Clinical	Triage, consultation, wards, maternity and theatre; viewing lab, imaging and pharmacy
Laboratory	Receiving lab requests and releasing results
Imaging	Receiving imaging orders and writing reports
Pharmacy	Dispensing prescriptions; viewing stock levels
Stores	Stock, purchasing, requisitions, returns and adjustment requests
Reports	Facility and Ministry of Health reports

The **administrator** has every screen. Some decisions stay with the administrator on purpose, so that the person who handles money or stock is not the same person who approves exceptions:

- Approving **stock adjustments** (expired, damaged, lost, or a count that does not match).
- Approving or rejecting **waivers** at the cashier.
- Managing users, departments, wards and prices.

Every action in Tunzuri is recorded with the name of the person who did it and the time.

A patient's journey

Most outpatient visits follow the same path. Tunzuri moves the patient from one queue to the next as each step is finished.

1. **Reception** registers the patient (or finds their record) and opens a visit. The patient joins the **Triage** queue.
2. **Triage** records vital signs, the chief complaint and a priority. The patient joins the **Consultation** queue.
3. **Consultation**: the clinician writes notes, records diagnoses, and orders lab tests, imaging and medicines. Each order sends the patient to that queue and adds the charge to the bill automatically.
4. **Laboratory** and **Radiology** enter and release results, which appear on the visit.
5. **Cashier** takes payment for cash patients. Insurance and corporate visits are billed to the scheme.
6. **Pharmacy** dispenses the medicines. A cash patient collects them once the bill is paid.
7. The visit is **completed** once the cash balance is settled and nothing is pending. If the patient needs a bed, the clinician **admits** them to a ward instead.

Patients are always seen in this order in every queue: emergencies first, then urgent, then by arrival time.

Reception

Registering a patient

1. Go to **Patients** and select **Register a patient**.
2. Fill in the surname, given name, sex, date of birth, phone, district and next of kin. National ID, village, blood group, known allergies and a photo are optional but useful.
3. Tick the consent box. The patient (or guardian) must agree to the facility keeping their health record under the Data Protection and Privacy Act, 2019.
4. Select **Save patient**. Tunzuri gives the patient a unique patient number.

Duplicate check. If the phone number, name and date of birth match an existing record, Tunzuri stops and shows the match. Open the existing record instead of creating a second one. Only if you are sure this is a different person, tick **Register anyway** and save again.

Known allergies show in red on every visit, lab request and prescription for that patient.

Finding a patient

The **Patients** screen (Patient directory) lets you search by name, phone number or patient number, and sort the list. It also shows totals: all patients, inpatients, records opened today and insured patients. Select a name to open the patient's record, with demographics, next of kin, a clinical brief, active orders and visit history.

To correct details, open the patient and select **Edit**.

Opening a visit

From the patient's record, use **Open a visit**:

Field	What to choose
Visit type	Outpatient or Emergency
Priority	Routine, Urgent or Emergency
Payer	Cash, Insurance or Corporate
Department	Where the patient is going
Insurance scheme and member number	Only for insurance or corporate visits
Chief complaint	The reason for the visit, in the patient's words

Select **Open visit**. The patient joins the triage queue.

Appointments

Go to **Appointments** to book and check in planned visits.

Booking for a registered patient

1. Choose **Already registered**.
2. Type the patient's name, phone or patient number and choose them from the matches.
3. Choose the department, the date and time (**When**) and a reason, then select **Book appointment**.

Booking for a new patient

Choose **New patient**. You can register the patient and book the visit in one step. Fill in the same details as for registration, tick consent, then choose the department and time.

On the day

- When the patient arrives, open **Check in**, choose who pays, and select **Open visit**. The patient joins the triage queue.
- If they do not come, select **No-show**. If the appointment is called off, select **Cancel**.

Queues

Queues shows every service point (Triage, Consultation, Laboratory, Radiology, Pharmacy, Cashier) and who is waiting.

1. Select **Call next to …** for your service point. Tunzuri picks the next patient by priority and arrival time.
2. Serve the patient.
3. Select **Done** when they leave your service point.

You can only call patients for service points you have access to.

Triage

Open the visit (from the queue or the patient's record). Under **Triage**, open **Record vital signs**:

1. Enter the vital signs, the chief complaint and any notes.
2. Set the **Priority**. Emergencies go to the front of every queue that follows.
3. Select **Save triage**. The patient moves to the consultation queue.

Consultation

Open the visit and work through the sections in order.

Notes and diagnosis

Open **Write consultation**, write your history, examination and plan, then search for and tick one or more ICD-10 diagnoses. Select **Save consultation**. Diagnoses feed the HMIS 105 report automatically.

Ordering lab tests

Under **Results** → **Order lab tests**, type to filter the test list, tick the tests and add clinical notes. Select **Order tests**. The patient joins the laboratory queue and each test is charged to the bill. You can **Cancel** a test that has not been resulted.

Ordering imaging

Under **Order imaging**, search for the study, add clinical notes, and select **Order imaging**.

Prescribing

Under **Prescriptions** → **Prescribe**:

1. Search for the medicine. The list shows its unit and price.
2. Enter the dose, frequency, number of days and quantity.
3. Select **+ Add another medicine** for more lines, or **Remove this medicine** to drop one.
4. Select **Prescribe**.

Prescribing does not move stock or charge the patient. The pharmacist does both when they dispense.

Admitting a patient

Under **Admit**, search for a free bed by ward or bed name, enter the working diagnosis, and select **Admit**. If no bed is free, beds being cleaned can be marked clean on the **Wards** page.

Completing the visit

The **Bill** section shows every charge. Select **Complete visit** when the patient is finished. Tunzuri only allows this once the cash balance is settled and no prescriptions or lab tests are still pending.

Laboratory

Laboratory lists every test waiting, emergencies first, then urgent, then oldest. The top of the screen shows tests waiting, urgent requests, the longest wait and tests verified today. Use the search box to find a patient or test.

For each test:

1. Check the patient, age, visit and any allergies shown.
2. Enter the result. The reference range is shown beside the box.
3. Select **Verify**.

Verifying releases the result to the clinician on the visit and sends the patient a text message. Reagents used by each test are taken from the laboratory store automatically, if the administrator linked a reagent to the test in the price list.

Radiology

Radiology lists imaging orders. Open an order, write the report, attach the image if you have one, and select **Save report**. The report appears on the visit.

Pharmacy

Dispensing a prescription

Pharmacy lists every prescription line waiting to be dispensed or collected. Each card shows the medicine, the patient, the dose, frequency and duration, how much was prescribed, already dispensed and still to dispense, and how much is on the pharmacy shelf.

1. Check the quantity to dispense. It starts at the amount still owed on the prescription. You can dispense part now and the rest later.
2. If you must give a different product, search for it under **Substitute medicine**. Leave it as *Dispense [medicine]* to give what was prescribed.
3. Select **Dispense**.

Dispensing takes stock from the pharmacy shelf, using the batch that expires first, and charges the patient at the price for their payer. A cash patient collects the medicine once the bill is paid at the cashier. Insured patients can collect straight away.

Cancel line removes a prescription line that will not be dispensed. A line that has already been dispensed cannot be cancelled.

Watching the pharmacy shelf

- **Running low on the pharmacy shelf** (amber box at the top) lists every medicine at or below its reorder level, with how many are left and how many the main store holds.
- **Pharmacy shelf** lists every batch on hand with its expiry date, quantity, price and reorder level. Select a medicine's name to open its full stock card.
- **All stock levels and history** (top right) opens the Stock levels screen.

Restocking the pharmacy

Under **Restock the pharmacy**:

1. The form is already filled with the low items and a suggested quantity, enough to bring each back to twice its reorder level. Change the quantities, remove lines, or select **Add another item**.
2. Select **Send requisition**. The main store sees it on the Inventory screen and issues the stock.

To send stock back, for example because it is overstocked or close to expiry, use **Return to the main store** below the requisition form. Enter the item, quantity and a reason. The batch and expiry date go back with the stock.

You need the Stores area (or the Pharmacy and stores role) to send requisitions and returns.

Cashier

Shifts

You must open a shift before taking money.

1. Enter the **Opening float** (cash in the drawer at the start) and select **Open shift**.
2. At the end of your shift, count the cash, enter it under **Cash counted in the drawer**, and select **Close shift**. Tunzuri records what you should hold (the float plus cash taken) and any difference.

Taking a payment

Bills waiting for payment lists every cash bill with a balance.

1. Choose the **Method**: Cash, MTN Mobile Money, Airtel Money or Card.
2. Check the **Amount**. It starts at the full balance; enter less for a part payment.
3. For mobile money or card, enter the transaction **reference**.
4. Select **Take payment**. Print the receipt from the receipt page.

Waivers

If a patient cannot pay, select **Patient cannot pay? Request a waiver**, enter the amount and reason, and select **Send for approval**. The administrator approves or rejects it under **Waivers**. The cashier cannot approve their own request.

Insurance and corporate accounts

Schemes

Add each insurer or corporate client under **Schemes**: name, kind, the cover limit per visit, and whether charges need an approved preauthorisation first. Patients on a scheme are registered with that scheme and their member number when their visit is opened.

Preauthorisations

Under **Request a preauthorisation**, search for the insured visit by patient name or visit number, describe the service, and enter the amount. Select **Request**. When the insurer replies, enter their reference and select **Approve** or **Reject**.

Claims

1. Under **Claims**, choose the scheme and the date range, then select **Compile claim**. Tunzuri gathers every charge for that scheme in the period.
2. Check the claim, then select **Submit to insurer**.
3. When payment arrives, enter the **Amount received** and **Remittance reference**, and select **Record remittance**.

Wards

The ward board

Wards shows every ward, its beds and the inpatients on it. A bed that was just vacated shows as *cleaning*; select **Mark clean** when it is ready for the next patient.

Caring for an inpatient

Select a patient to open their admission.

- **Nursing notes**: write a note and select **Save note**.
- **Medication chart**: search for the medicine, enter the dose and status, and select **Chart dose**. Charting does not move stock. The stock already left when the pharmacy dispensed it.
- **Ward stock used**: when you use supplies from the ward store, such as a cannula or a giving set, choose the store, search for the item, enter the quantity and select **Issue**. Billable items are charged to the patient. Non-billable items, such as gloves and cotton wool, are recorded as a cost to the ward.

Discharge

Under **Discharge**, write the discharge summary, choose the outcome, and select **Discharge**. As with completing a visit, the cash balance must be settled and nothing may be pending. After discharge the record is closed and the bed goes to *cleaning*.

Maternity

- **Antenatal visit**: enter the mother's patient number (you can type her name and pick from the suggestions), gestation in weeks, fundal height, fetal heart rate, blood pressure and notes, then select **Save antenatal visit**.
- **Delivery register**: enter the mother's patient number, the delivery time, mode, outcome, baby's sex, birth weight and Apgar score, then select **Save delivery**.

Recent antenatal visits and deliveries are listed below the forms.

Theatre

1. Under **Book theatre**, enter the patient number, procedure and time, and select **Book**.
2. After the operation, find the booking in the list, write the findings and operation note, and select **Save operation note**.

Inventory and stock

Tunzuri keeps one stock ledger for the whole facility. Every unit that comes in or goes out is recorded with the item, batch, store, quantity, reason, the patient or department, and the person who did it. Stock on hand in any store is always the total of its movements, so nothing changes without a record.

How stock moves

Each facility starts with these stores: **Main store**, **Pharmacy**, **Laboratory**, **Female ward store**, **Male ward store** and **Theatre store**. New wards can have their own store.

1. Stock arrives in the **main store**, from a supplier delivery, a donation, or as an opening balance.
2. Department stores **request** stock from the main store (a requisition), and the storekeeper **issues** it.
3. Departments **use** stock: the pharmacy dispenses to patients, wards issue to inpatients, and the laboratory uses reagents for tests.
4. Unwanted stock is **returned** to the main store. Losses, damage, expiry and count differences are **adjusted** with the administrator's approval.

Setting up your items

A new facility has no stock items. There are three ways to add them, all under **Inventory** → **Items and stock**.

Starter list (fastest). Open the **Starter list** tab and select **Load the starter list**. Tunzuri adds about 35 medicines, supplies and lab reagents that most facilities stock, with suggested prices and reorder levels. Items you already have are skipped, so it is safe to run again. Check the prices afterwards under **Price list**.

One item at a time. Open the **New item** tab:

Field	Meaning
Name and strength	For example *Amoxicillin 250mg*
Counted in	The unit you count and dispense: tablet, bottle, vial, pair
Code (optional)	Leave blank and Tunzuri makes one from the name
Category	Medicine, Consumable or Lab reagent
Billing	Billable medicine, billable consumable, or non-billable (a department cost). Set automatically from the category; change it if needed
Cash price to patient	The price a cash patient pays per unit
Reorder when stock falls to	The level at which the item shows as low

If you already have some on the shelf, fill in **Opening stock** on the same form: quantity, store, batch number, expiry date and unit cost.

Import from a spreadsheet (best for a long list). Open the **Import from CSV** tab.

1. Select **Download the template**.
2. Fill it in with Excel or Google Sheets, one row per item, then save it as **CSV**.
3. Choose the file and select **Import**.

Column	Required	Notes
name	Yes	Name and strength
unit	Yes	tablet, bottle, pair…
category	No	medicine, consumable or reagent (default medicine)
billing_type	No	billable_medicine, billable_consumable or non_billable
cash_price	No	Price per unit in UGX
reorder_level	No	Low-stock level
sku	No	Your own code; made from the name if blank
store	No	Store name exactly as in Tunzuri; default Main store
quantity	No	Opening stock. Leave blank to add the item only
batch_number	No	Made up automatically if blank
expiry_date	No	Written as YYYY-MM-DD, for example 2027-06-30
unit_cost	No	What you paid per unit, in UGX

An item that already exists, matched by name or code, is not created again; only its stock is added. So you can also use an import for a large stock top-up. If any row has a mistake, nothing is saved and Tunzuri lists the rows to fix, for example *Row 3: The unit field is required.* Fix the file and upload it again.

Adding stock without a purchase order

Open the **Add stock** tab when stock arrives outside the purchase-order process, or when you are entering what is already on your shelves.

1. Search for the item and choose the store.
2. Under **Where it came from**, choose *Opening balance*, *Delivery without a purchase order* or *Donation*.
3. Enter the quantity and, if you have them, the batch number, expiry date and unit cost.
4. Use **Notes** for the supplier, donor or delivery note number.
5. Select **Add stock**.

Buying from suppliers

Placing a purchase order

1. Under **Purchase orders**, search for the supplier, or select **Add a new supplier** and type the name.
2. Search for the item, enter the quantity and unit cost, and select **Send order**.

Receiving goods

When the delivery arrives, find the order and, for each line, enter the batch number, expiry date and quantity received. Select **Receive**. The stock goes into the main store. If only part of an order arrives, receive what came. The order stays open until everything is received.

Requisitions: moving stock to a department

Any department store can ask the main store for stock.

1. Under **Requisitions from the main store**, choose the store the stock is for.
2. Select **Fill from reorder levels** to add every item that store is low on, with suggested quantities. Or search for items yourself, using **Add another item** for each one.
3. Select **Send requisition**.

The storekeeper sees open requisitions at the top of the list, with *issued X of Y* for each line, and selects **Issue**.

- If the main store has enough, everything moves and the requisition shows *Issued*.
- If the main store is short, Tunzuri issues what is there and marks the requisition *Partial*. When new stock arrives, select **Issue the rest**.
- If the main store has none of the items, nothing moves and Tunzuri says so.

Moving stock between stores never charges a patient.

Returning stock

Use **Return stock to the main store** for stock a department no longer needs. Choose the store, item and quantity, and give a reason, for example *overstocked* or *near expiry*. Batches and expiry dates go back with it.

Adjustments: expiry, damage, loss and counts

When stock on the shelf does not match Tunzuri:

1. Under **Stock adjustments**, choose the store and item.
2. Enter the **Change in quantity**: a minus number to remove (for example –5), a plus number to add.
3. Choose the reason (*Expired*, *Damaged*, *Lost*, *Count did not match*) and add notes.
4. Select **Request adjustment**.

Stock does not change until the administrator selects **Approve**. The approval is recorded on the stock card.

Stock levels

Stock levels shows every item and how many are in each store, the total, its reorder level, how many were used in the last 30 days, and the nearest expiry date. Each item has a status:

Status	Meaning
In stock	Nothing needs attention
Low	The total is at or below the reorder level. Time to order
Expiring soon	A batch expires within 90 days. Use it first or return it
Has expired stock	A batch is past its expiry date. Remove it with an adjustment
Out of stock	None in any store

Search for an item by name, or tick **Only items that need attention** to see just the items to act on.

The stock card: an item's full history

Select any item to open its stock card.

- **On hand now** lists every batch in every store, with its expiry date and unit cost.
- **Stock card** lists every movement in and out, newest first. Each row shows when it happened, the store, what happened, the quantity in or out, the balance left afterwards, the batch, the department, the patient (for dispenses and ward issues), who did it (and who approved it, for adjustments), and the document it belongs to (requisition, goods receipt or prescription).

Filter by store and dates to answer questions like "where did the 200 tablets issued to the pharmacy last week go?"

Stock usage: what each department and employee used

Stock usage totals what was used up in a period: dispensed, issued on the wards, used by a department, or used by the laboratory. Transfers between stores are not counted as use.

1. Choose the dates (**From**, **To**).
2. Under **Group by**, choose **Department**, **Employee** or **Item**.
3. Narrow it if you need to by kind of use, department, employee or item. Choose *Adjustment* to see write-offs, or *Returned to main store* to see returns.
4. Select **Update**.

Each group shows the quantity of each item and its value at cost, with the total value at the top. Select **Export CSV** to open the same figures in Excel.

Reports

Reports builds the facility's figures from daily work, for the dates you choose:

- **Cash collected** in the period.
- **Dispenses without a charge**: should always be zero. Anything else needs checking.
- **Ward issues in 7 days.**
- **HMIS 105 outpatient attendance**: new and repeat attendances by sex and age band, plus diagnoses by ICD-10 code.
- **HMIS 108 inpatient**: admissions, discharges, deaths, bed days and bed occupancy.

Select **Export HMIS CSV** to download the figures for the Ministry of Health return.

Administration

These screens are for the facility administrator.

Users

Users lists everyone who can sign in. Search by name or email.

Adding a user

1. Select **Add user**. Enter the name, email, phone and a temporary password.
2. Choose **Administrator, full access**, or **Chosen areas**.
3. For chosen areas, pick the person's **Department**. Tunzuri ticks the areas that department normally uses. Add or remove ticks as needed.
4. Select **Add user**. Give the person their email and temporary password, and ask them to change it when they first sign in.

To change someone's access, or to stop them signing in when they leave, open their account, change the details or set them inactive, and select **Save changes**. Deactivate accounts rather than sharing them. Every action is recorded against a person.

Departments

Add departments with a name, code and kind. The kind decides which areas are ticked when you put someone in that department.

Wards and beds

Wards & beds lets you:

- Add a ward with a name, short code, department and number of beds. Tick **Also create a ward stock store** so the ward can hold and request its own supplies.
- Grow or shrink a ward, rename beds (for example *Cot 3* or *Side room*), and close beds that are out of service.
- Remove a bed that was never used. A bed with admission history can only be closed.

Price list

Price list holds everything the facility charges for:

- **Services**: consultation, registration, bed day and so on, with a code, name, category and cash price.
- **Stock items**: medicines and supplies (see *Setting up your items*).
- **Lab tests**: priced through a linked service. Add the service first. You can also set the specimen, result unit, reference range, and the reagent used by each test, so reagent stock goes down automatically.
- **Imaging studies**: priced through a linked service.

Audit trail

Audit trail lists every recorded action: who did what, and when. Use it to answer questions about a change to a record, price or user.

Setting up a new facility

When a facility signs up, its owner confirms their email and goes through a short setup wizard:

1. **Facility details**: name, type, district, address, phone, email and MoH facility code. These appear on receipts, reports and patient documents.
2. **Wards & beds**: two wards are created to start. Rename them, change the number of beds, or add more.
3. **Ready to go live**: a summary of what the facility starts with. Select **Go live**.

Every new facility starts with standard departments, the six stock stores, two wards and starter prices for registration, consultation and bed days.

Suggested first-week checklist for the administrator

1. Add your staff under **Users** and give each person their areas.
2. Check **Departments** and **Wards & beds**.
3. Set your prices under **Price list**: services, lab tests and imaging studies.
4. Add stock items: load the **Starter list** or import your own list (*Inventory → Items and stock*).
5. Count what is on your shelves and enter it as **opening balances**, in the store where it actually is.
6. Set realistic **reorder levels** so Stock levels and the pharmacy warn you in time.
7. Add your insurance schemes and corporate clients.

Platform administrator

The platform administrator runs Tunzuri for all facilities. They never see patient records.

- **Platform overview**: facility counts, new sign-ups, visits, bed occupancy and facilities that need attention.
- **Facilities**: search and sort facilities; **Provision facility** to create one with its first administrator and a temporary password (it goes live straight away); **Deactivate** or **Activate** a facility.
- **Administrators** (per facility): add another administrator or switch one off.

Troubleshooting and common questions

I cannot see a screen I need. Your account does not have that area. Ask your administrator.

I was signed out. Tunzuri signs you out after 15 minutes without activity. Sign in again.

The pharmacy says there is not enough stock, but the main store has plenty. Dispensing only uses stock on the pharmacy shelf. Send a requisition to the main store (*Restock the pharmacy*) and ask the storekeeper to issue it.

A requisition shows "Partial". The main store did not have enough. Once stock arrives, the storekeeper selects **Issue the rest**.

The stock in Tunzuri does not match the shelf. Count again. If it still differs, request an adjustment with the reason *Count did not match*. Use the item's stock card to trace where the difference started.

I cannot complete a visit or discharge a patient. The cash balance is not settled, or a prescription or lab test is still pending. Take payment, dispense or cancel the prescription, and release or cancel the lab test.

The CSV import was rejected. Nothing was saved. Fix the rows listed in the message and upload the whole file again. Common causes: a missing name or unit, a store name spelled differently from Tunzuri, or an expiry date not written as YYYY-MM-DD.

There are no items to choose from. No stock items have been added yet. Someone with the Stores area, or the administrator, adds them under *Inventory → Items and stock*.

A patient was registered twice. Ask your administrator. Do not keep using both records.

Glossary

Term	Meaning
Batch	One lot of stock with its own batch number, expiry date and cost
Billable / non-billable	Whether using an item charges the patient or is a department cost
Opening balance	Stock already on the shelf when you start using Tunzuri
Payer	Who pays for a visit: cash, insurance or corporate
Preauthorisation	The insurer's approval before a service is given
Reorder level	The quantity at which an item shows as low
Requisition	A department's request for stock from the main store
Stock card	An item's full history of movements, with the balance after each one
Stock ledger	The single record of every stock movement in the facility
Waiver	Part or all of a bill forgiven, with the administrator's approval